

Forward Arrow Services

Clinic AI Use Case Definition Checklist

1. Define the Workflow Problem

- ☐ Which workflow is creating the most friction right now? (scheduling, intake, charting, billing, prior auth, patient messages)
- ☐ Which staff role feels this most? (front desk, MA, provider, billing, manager)
- ☐ Where are delays, duplicate work, or manual re-entry happening today?
- ☐ Is this hurting staff time, patient experience, or cash flow?

2. Define the Target Outcome

- ☐ What would “better” look like in daily operations?
- ☐ What metric should improve? (faster turnaround, fewer callbacks, cleaner claims, less after-hours charting)
- ☐ Would staff actually feel the improvement within 30–60 days?
- ☐ Is this solving a real bottleneck, not just adding another tool?

3. Identify a Real Use Case

- ☐ Is this repetitive enough to justify automation or AI assistance?
- ☐ Does it rely on structured data the practice already has?
- ☐ Can AI assist the task without replacing provider judgment?
- ☐ Can this be piloted safely in one workflow first?

4. Risk & Compliance Check

- ☐ Will this touch PHI, PII, or PCI data?
- ☐ Is the tool approved for clinical or administrative use?
- ☐ Could staff be tempted to paste sensitive data into a non-approved tool?
- ☐ Is human review required before anything is used in patient care, billing, or communication?

5. Human Oversight

- ☐ Who reviews the output before action is taken?
- ☐ Who owns the risk if the output is wrong?
- ☐ Are staff trained not to trust the tool blindly?
- ☐ Is there a stop/escalation process if something looks off?

6. Pilot First

- ☐ Can this start with one provider, one role, or one workflow?
- ☐ Do we know what success and failure look like before starting?
- ☐ Are we measuring results during the pilot?
- ☐ Is there a plan to refine the process before scaling?

7. Measure Operational Impact

- ☐ Are we comparing against a real baseline?
- ☐ Is time actually being saved?
- ☐ Are errors, rework, or delays going down?
- ☐ Is this improving operations, or just creating more oversight work?

8. Boundaries & Guardrails

- ☐ What should AI never be used for in this practice?
- ☐ Where must human review remain mandatory?
- ☐ Are staff boundaries documented clearly?
- ☐ Are escalation paths defined when output is uncertain or wrong?

Final Readiness Check

- ☐ Real workflow problem identified
- ☐ Measurable outcome defined
- ☐ Risks understood before pilot
- ☐ Human oversight assigned
- ☐ Pilot scope ready